

**Laboratory Investigation Report**

Patient Name	: Amita Mishra	Centre	: 1534 - Arctern Healthcare Noida
Age/Gender	: 36 Y 9 M 16 D /F	OP/IP No	: OP/145484
Max ID/Mobile	: 284895/8752005200	Collection Date/Time	: 22/May/2022 08:56AM
Lab ID	: 1266052200460	Receiving Date	: 22/May/2022
Ref Doctor	: Dr.Dr. Richika Sahay Shukla	Reporting Date	: 22/May/2022
Passport No.	:		

Test Name	Serology Result	Unit	Bio Ref Interval
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Rapid Card Test - HCV*, Serum

Immunochromatography

HCV Card Test Negative

Comment Interpretation

It is only a screening test. All reactive samples should be confirmed by HCV quantitative PCR

A non reactive result does not exclude the possibility of exposure to or infection with HCV.

Patients with auto immune liver disease may show falsely reactive results

HIV I & II*, Serum

CLIA

HIV (I and II) Negative

Interpretation

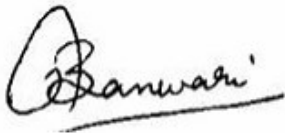
1) A Negative result implies that no Anti HIV-1 or HIV-2 antibodies have been detected in the sample by this method. This means that either the patient has not been exposed to HIV-1 or HIV-2 infection or the sample has been taken during the "WINDOW PERIOD" (before the development of detectable levels of antibodies).

2) Positive result suggests the possibility of HIV-1 or HIV-2 infection.

Advise:

To rule out false positivity, false negativity and window period, kindly perform "Confirmatory Tests" like HIV I RNA (Qualitative) Real Time PCR.

Kindly correlate with clinical findings

***** End Of Report *******Dr. Akash Banwari, M.D.(Path)**
Pathologist

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Clinical Biochemistry

Kidney Function Test (KFT) Profile with Calcium, Uric Acid, Serum

Date	22/May/2022 08:56AM	Unit	Bio Ref Interval
Urea Urease, UV	13.0	mg/dL	17.0 - 43.0
Blood Urea Nitrogen Calculated	6.07	mg/dL	7.9 - 20.0
Creatinine Alkaline picrate kinetic	0.54	mg/dL	0.6 - 1.1
eGFR MDRD	127.18	ml/min/1.73 m ²	
Bun/Creatinine Ratio Calculated	11.24	Ratio	12:1 - 20:1
Uric Acid Uricase, Colorimetric	2.7	mg/dL	2.6 - 6.0
Calcium (Total) Arsenazo III	9.03	mg/dL	8.8 - 10.6
Sodium ISE indirect	137.8	mmol/L	136 - 146
Potassium ISE indirect	3.7	mmol/L	3.5 - 5.1
Chloride ISE indirect	105.2	mmol/L	101 - 109
Bicarbonate Enzymatic	23.5	mmol/L	21 - 31

Interpretation Ref. Range

eGFR - Estimated Glomerular Filtration Rate is calculated by MDRD equation which is most accurate for GFRs ≤ 60 ml / min / 1.73 m². MDRD equation is used for adult population only.

<60ml / min / 1.73 m² - Chronic Kidney Disease

<15 ml / min / 1.73 m² - Kidney failure

BUN/Creatinine Ratio :-

Increased in reduced renal perfusion (e.g. dehydration, Hypovolemic shock, Congestive Heart Failure) or Obstructive uropathy. Decreased in Acute Renal Tubular necrosis.



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Clinical Biochemistry

Liver Function Test (LFT), Serum

Date	22/May/2022 08:56AM	Unit	Bio Ref Interval
Total Protein Biuret	7.75	g/dL	6.6 - 8.3
Albumin Bromocresol Green (BCG)	4.2	g/dL	3.5 - 5.2
Globulin Calculated	3.5	g/dl	2.3 - 3.5
A.G. ratio Calculated	1.2		1.2 - 1.5
Bilirubin (Total) DPD	0.50	mg/dL	0.3 - 1.2
Bilirubin (Direct) Diazotization	0.11	mg/dL	0.0 - 0.2
Bilirubin (Indirect) Calculated	0.39	mg/dL	0.1 - 1.0
SGOT- Aspartate Transaminase (AST) UV without P5P	20	U/L	< 35
SGPT- Alanine Transaminase (ALT) UV without P5P	10	U/L	< 35
AST/ALT Ratio Calculated	2.00	Ratio	
Alkaline Phosphatase PNPP, AMP Buffer	78	U/L	30 - 120
GGTP (Gamma GT), Serum Enzymatic Rate	13.0	U/L	< 38

Interpretation AST/ALT Ratio :-

In Case of deranged AST and/or ALT, the AST/ALT ratio is < 2.0 in alcoholic liver damage and > 2.0 in non – alcoholic liver damage



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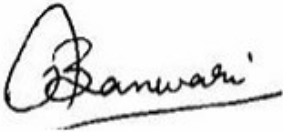
Clinical Biochemistry**Random Blood Sugar, RBS (Glucose), Fluoride Plasma**

Date	22/May/2022 08:56AM	Unit	Bio Ref Interval
Random Glucose Hexokinase	89	mg/dL	74 - 140

Interpretation A random plasma glucose of > 200 mg/dl with symptoms of diabetes is diagnostic. A confirmatory fasting plasma Glucose (FPG) test or OGTT should be completed on a different day if the clinical condition of the patient permits.

Kindly correlate with clinical findings

*** End Of Report ***



Dr. Akash Banwari, M.D.(Path)
Pathologist



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Clinical Pathology

Urine Routine And Microscopy

Date	22/May/2022 07/Apr/21	Unit	Bio Ref Interval
	08:56AM 11:52AM		

Macroscopy

Methyl red bromothymol blue(Strip Method)

Colour	PALE YELLOW	Colorless		Pale Yellow
PH	6.0	6.0	..	5-6
Specific Gravity	1.010	1.002		1.015 - 1.025
Protein	Neg	Nil		Nil
Glucose.	Neg	Nil		Nil
Ketones	Neg	Nil		Nil
Blood	Neg	Nil		Nil
Bilirubin	Neg	Nil		Nil
Urobilinogen	Normal	Normal		Normal
Nitrite	Neg	Negative		

Microscopy

Light Microscopy/Image capture microscopy

Red Blood Cells (RBC)	0	Nil	/HPF	Nil
White Blood Cells	0	0 - 1	/HPF	0.0-5.0
Squamous Epithelial Cells	27	Nil	/HPF	
Cast	Nil	Nil	/LPF	Nil
Crystals	Nil	Nil	..	Nil
Bacteria	Nil	Nil	/HPF	Nil

Kindly correlate with clinical findings

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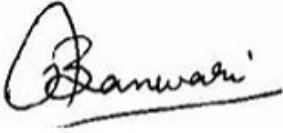
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Clinical Pathology

Dr. Akash Banwari, M.D.(Path)
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Hematology

Activated Partial Thromboplastin Time (APTT) Test, Sodium Citrate

Date	22/May/2022	Unit	Bio Ref
	08:56AM		Interval
APTT	31.300	Sec	24.9 - 33.3
Photo-Optical-Nephelometry			
Control	29.0	sec	

Interpretation

(Activated Partial Thromboplastin Time; PTT)

APTT is the test which checks the "intrinsic coagulation" pathway and is useful for detecting screening of haemophilia A & B, screening of coagulation inhibitors like lupus anticoagulant.

APTT can also be used to monitor heparin monitoring.

Raised APTT is found in - Liver disease, DIC, heparin treatment, circulating inhibitors (Lupus Anticoagulant), and haemophilia (deficiency of Factor VIII & IX).

Low APTT may be seen in - high concentration Factor VIII and is independent risk factor for increased incidence of thrombus formation.

Advice: - 'APTT mixing study', 'Lupus Anticoagulant test', 'specific Factor(s) assay' may be added on for further evaluation.

Prothrombin Time (PT-INR), Sodium Citrate

Date	22/May/2022	Unit	Bio Ref
	08:56AM		Interval
Prothrombin Time (PT)	11.300	Sec	12.1 - 15.1
Photo-Optical-Nephelometry			
MNPT Value	11.4	Sec	
INR	0.991		

Interpretation

(Syn: - Prothrombin Time)

PT is the test which checks the "extrinsic coagulation" pathway and is useful for detecting coagulation deficiency, liver disease and disseminated intravascular Coagulation (DIC).

PT can also be expressed as International normalized ratio (INR) used for monitoring warfarin therapy.

Raised PT value seen in - factor deficiency (Fibrinogen (I), Prothrombin (II), factor V, VII, X), oral anticoagulation therapy, liver diseases, Vitamin K deficiency and DIC.

Advice: - 'PT mixing study', 'specific factor(s) assay' may be added on for further evaluation.

CBC (Complete Blood Count), Whole Blood EDTA

Date	22/May/2022	23/Jul/21	07/Apr/21	Unit	Bio Ref Interval
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Hematology

	08:56AM	07:21AM	11:52AM		
Haemoglobin	10.2	10.4	7.8	g/dl	12.0 - 15.0
Modified cyanmethemoglobin					
Packed Cell, Volume	33.0	32.6	27.4	%	40-50
Calculated					
Total Leucocyte Count (TLC)	6.4	7.0	7.7	10~9/L	4.0-10.0
Electrical Impedance					
RBC Count	4.59	4.31	3.80	10~12/L	3.8-4.8
Electrical Impedance					
MCV	71.9	75.5	71.9	fL	83-101
Electrical Impedance					
MCH	22.1	24.1	20.4	pg	27-32
Calculated					
MCHC	30.7	31.9	28.4	g/dl	31.5-34.5
Calculated					
Platelet Count	158	155	177	10~9/L	150-410
Electrical Impedance					
MPV	12.1	13.8	11.6	fl	7.8-11.2
Calculated					
RDW	17.9	18.1	21.1	%	11.5-14.5
Calculated					

Differential Cell Count

VCS / Light Microscopy

Neutrophils	50.5	52.9	65.5	%	40-80
Lymphocytes	41.5	37.2	27.1	%	20-40
Monocytes	7.5	8.9	6.5	%	2-10
Eosinophils	0.1	0.7	0.5	%	1-6
Basophils	0.4	0.3	0.4	%	0-2

Absolute Leukocyte Count

Calculated from TLC & DLC

Absolute Neutrophil Count	3.23	3.7	5.04	10~9/L	2.0-7.0
Absolute Lymphocyte Count	2.7	2.6	2.1	10~9/L	1.0-3.0
Absolute Monocyte Count	0.48	0.62	0.5	10~9/L	0.2-1.0
Absolute Eosinophil Count	0.01	0.05	0.04	10~9/L	0.02-0.5
Absolute Basophil Count	0.03	0.02	0.03	10~9/L	0.02-0.1



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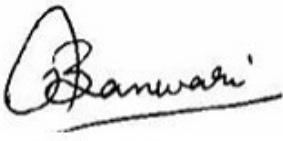
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Lab ID	: 1266052200460	Receiving Date	: 22/May/2022
Ref Doctor	: Dr.Dr. Richika Sahay Shukla	Reporting Date	: 22/May/2022
Passport No.	:		

Immunoassay

Thyroid Stimulating Hormone (TSH) - Ultrasensitive, Serum

Date	22/May/2022 07/Apr/21	Unit	Bio Ref Interval
	08:56AM 11:50AM		
Thyroid Stimulating Hormone	1.00 3.76	µIU/mL	0.34 - 5.6
CLIA			

Interpretation

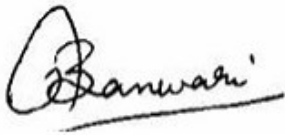
Parameter	Unit	Premature (28 - 36 Weeks)	Cord Blood (> 37 weeks)	Upto 2 Month	Adult	1st Trimester	2nd Trimester	3rd Trimester
TSH	uIU/ml	0.7 - 27.0	2.3 - 13.2	0.5 - 10	0.38 - 5.33	0.1 - 2.5	0.2 - 3.0	0.3 - 3.0

Increased in primary Hypothyroidism.
Decreased in primary Hyperthyroidism

Note : TSH levels are subject to circadian variation, reaching peak levels between 2 – 4 am and at a minimum between 6 – 10 pm. The variation is of the order of 50% - 206 %, hence time of the day has influence on the measured serum TSH concentrations.

Kindly correlate with clinical findings

*** End Of Report ***



Dr. Akash Banwari, M.D.(Path)
Pathologist



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Phone: +91-124-6623 000 | (CIN No.: U85100DL2021PLC381826)

Helpline No. 7982 100 200 | www.maxlab.co.in | feedback@maxlab.co.in

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Laboratory Investigation Report

Patient Name	: Amita Mishra	Centre	: 1534 - Arctern Healthcare Noida
Age/Gender	: 36 Y 9 M 16 D /F	OP/IP No	: OP/145484
Max ID/Mobile	: 284895/8752005200	Collection Date/Time	: 22/May/2022 08:56AM
Lab ID	: 1266052200460	Receiving Date	: 22/May/2022
Ref Doctor	: Dr.Dr. Richika Sahay Shukla	Reporting Date	: 22/May/2022
Passport No.	:		

Serology

Test Name	Result	Unit	Bio Ref Interval
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Hepatitis B Surface Antigen,Serum

CLIA

HBsAg Test Value 0.38

Ref. Range

Negative	< 0.90
Borderline	0.90 - 5.0
Positive	> 5.0

Interpretation

- This test is used to detect hepatitis B surface antigen (HBsAg) in serum sample based on VITROS immunometric immunoassay technique and aid in the laboratory diagnosis of HBV infection.
- Viral hepatitis is a major public health problem with an estimated 257 million persistent carriers of hepatitis B virus (HBV) worldwide. Infection with HBV results in a wide spectrum of acute and chronic liver diseases that may lead to cirrhosis and hepatocellular carcinoma.
- Transmission of HBV occurs by percutaneous exposure to blood products, needle stick injury, sexual contact and perinatally from HBV-infected mothers to baby.
- Hepatitis B surface antigen (HBsAg), derived from the viral envelope, is the first antigen to appear following infection.
- Positive results should be correlated with other potential laboratory abnormalities and clinical picture.
- A negative test result does not exclude the possibility of exposure to or infection with hepatitis B virus.
- Levels of HBsAg may be undetectable both in early infection and late after infection.
- In rare cases HBsAg tests do not detect certain HBV mutant strains.
- HBs Ag disappears with recovery from clinical disease in most patients, however, it persists for years in carriers.



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**Laboratory Investigation Report**

Patient Name	: Amita Mishra	Centre	: 1534 - Arctern Healthcare Noida
Age/Gender	: 36 Y 9 M 16 D /F	OP/IP No	: OP/145484
Max ID/Mobile	: 284895/8752005200	Collection Date/Time	: 22/May/2022 08:56AM
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Test Name	Serology Result	Unit	Bio Ref Interval
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VDRL/RPR, Serum

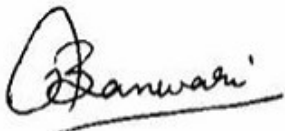
RPR(Syphilis) Non Reactive
Slide Flocculation

Comment**Interpretation**

1. It is a screening test for syphilis which is useful for following the progression of disease and response to therapy. Rising titers are of immense value in confirming the diagnosis.
2. Biological false positive reactions exhibit low titers and are seen in conditions like Viral fevers, Mycoplasma infection, Chlamydia infection, Malaria, Immunizations, Pregnancy, Autoimmune disorders & past history of Treponemal infection.
3. It is advisable to confirm the diagnosis by tests such as TPHA & FTA-ABS.
4. A definitive clinical diagnosis should not be made by result of a single test only, but should be made by taking clinical history and other laboratory findings in to account.

Kindly correlate with clinical findings

*** End Of Report ***



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